

إسم المريض ربيعاً : سميحة موسى سعد الله اسكندر
المحافظة : سوهاج الحي / المدينة : دار السلام
رقم البطاقة : 26907102601801 رقم التليفون : 01284156485
العمر(تاريخ الميلاد) : 10-07-1969 المهنة : ربه منزل
العنوان : دار السلام القاهرة
تاريخ تسجيل الإستمارة : 04-10-2026

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج: أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج:** الاورام

التعليق على التشخيص : -----

GR1-E
روستاقی کمیٹی
روستاقی مجلس
صحة البیابان المرفقة
للجنة و مدین

الاسم: د/محمد کیلانی

الاسم: د/ندی حسن

الاسم: د/نرمين رمضان

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس

وتلتزم جهة العلاج بلائحة المجالس الطبية.

كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

----- / د ----- / د ----- / د. | ----- / د. |

التشخيص النهائي :
 خطة العلاج :
 المستشفى :
 قسم :
 نفقات :
 ملاحظات :

ختم
شعار الجمهورية

**مدير عام
مديرية الشؤون الصحية**

**رئيس
المجلس الطبي المتخصص**

--

Name: سميحة موسى سعد الله اسكندر National ID: 26907102601801	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تعريف شخصية

تسوية
موسى سعد الله إسماعيل

١٢ من جمادى صفر ١٤٣٥ هـ
دار السلام - القاهرة

٢ ٦٩ ٠ ٧ ١ ٠ ٢٦ ٠ ١٨ ٠ ١

LB7411795

سوار بطن و حوض

سوار ماموجرام

سلسع

Name: سميحة موسى سعد الله اسكندر		
MR. : 1216		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالتها		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انها The patient was diagnosed with breast carcinoma that is estrogen receptor (ER) and progesterone receptor (PR) positive and HER2 negative. The patient underwent upfront left modified radical mastectomy (MRM) in January 2025, pathology revealing a pT2 N0 tumor with 0 of 6 lymph nodes involved. Adjuvant chemotherapy was administered, followed by adjuvant endocrine therapy with an aromatase inhibitor (AI) initiated in May 2025, specifically letrozole (Femara) with supportive care. The patient continues on Femara plus supportive care, having been on this regimen for three months as of May 2026. Surveillance imaging was reported as BIRADS 2. The next clinic visit is scheduled for 23 September 2026.		
تحريرا في ,		
	التاريخ :	19-09-2026
	الوقت :	
	هنا اسامه عبد الرحيم عبد البر	الطبيب المعالج :
يعتمد ,		



Ministry of Health, Government of India



1- Patient ID
2- Name

MDT Form

3- Medical History
4- Physical Examination

5- Investigations
6- Histology

7- Pathology
8- Staging

9- Treatment
10- Follow up



Family History

Phylogeny

Genetics

Site

Reason of MDT

Medical History

Age

Group

Date of MDT

Current Status

Presenting Complaint

Final Stage

MDT Date

Breast Imaging

1- Mammography
2- Ultrasound
3- MRI
4- PET/CT

Clinical data

Notes

pathology breast histology, ER, PR, Ki67, HER2, IHC, FISH, etc.

BA

ER

PR

Her 2 Neu

Stain

Ki67

ILC

HER

ILC

0

2%

pt 2/11

N

0

M

0 by pet

Sonography

Patient History

1- modified radical mastectomy free surgical margin
LN 0/21

MDT Outcome & Plan

- 1- adjuvant TAXOTERE ENDOXAN X1
- 2- ADJUVANT AIX5 YEARS

MEDICAL ONCOLOGY

SURGERY

Psychiatry

SIGNATURE

RADIOLOGY

PATHOLOGY

Neurology



Patient Name	: الأستاذة / منيرة موسى عبدالعظيم	Request No.	: 13009604
Patient No.	: 26907102601801	Request Date	: 04/02/2025
Path No.	: SR-207/25	Verification Date	: 09/02/2025
Sex	: Female	Financial Category	: برنامج دار السلام - نفقة الدولة
Age	: 55 Y	Department	: Out Patient
Referring Doc	:	Registered by	: marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer.

Nature of Specimen: Slide Revision

Gross Examination:

- 16 paraffin blocks labeled by (926303) and 24 slides were received.

Microscopic Examination:

MRM specimen:

- * Histological type: Multicentric mixed invasive duct and lobular carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade II (3+2+1)
- * DCIS: Focal DCIS 5 % (non comedo), focal LCIS 10%.
- * Angioinvasion: No.

* Perineural invasion: No.

* Skin involvement: Free nipple and areola.

* Lymph node involvement: Free (0/20).

* Core:

Histological type: Invasive duct carcinoma, focally necrotized.

* Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade III (3+3+2)

* IPT:

- ER: Strong nuclear reaction in 90% of tumor cells (score 8 /8).

- PR: Faint nuclear reaction in 5% of tumor cells (score 3 /8).

- Her2: No membrane reaction in the tumor cells (score 0).

- Ki67: 25% nuclear reaction of tumor cells.

Diagnosis:

Left breast cancer, Slide revision:

MRM specimen:

Verified by :

DR.Menna Nabil

Printing Date : 24/02/2025 10:12:56

Page 1 of 2

Printing By : marwa mohamed youss



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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name : الأستاذة / مديحة موسى عبدالله اسكندر
Patient No. : 26907102601801
Path No. : SR-207/25
Sex : Female
Age : 55 Y
Referring Doc :
Request No. : 13009604
Request Date : 04/02/2025
Verification Date : 09/02/2025
Financial Category : نفقة الدولة - اورام دار السلام
Department : Out Patient
Registered by : .marwa mohamed youss

Pathology Report

Multicentric mixed invasive duct and lobular carcinoma GII with focal DCIS and focal LCIS.
Free nipple and areola.

Free lymph nodes (0/20).

Core: Invasive duct carcinoma GIII, focally necrotized.

IPT:

- ER: Positive (score 8 /8).
- PR: Positive (score 3 /8).
- Her2: Negative (score 0).
- Ki67: Positive In 25% of tumor cells

Signature
Prof. Dr. Lubna Omar.
Dr. Menna Nabil.

Verified by :
DR.Menna Nabil

Printing Date : 24/02/2025 10:12:56

Page 2 of 2

Printing By : .marwa mohamed youss

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Dr.

Elia Anis Ishak

M. Sc. Ph. D. Pathology - Prof. of Pathology, Cairo University
Fellow, Armed Forces Institute of Pathology, USA
Member, United States Canadian Academy of Pathology

Dr.

Shady Elia Anis

M. Sc. MD Pathology, Ass. Prof. of Pathology, Cairo University
Fellow, Mass General Hospital, Harvard, USA
Member, United States Canadian Academy of Pathology

Patient Name : سميرة موسى سيد الله

Age : 55 Y : 6 M

Referred By Prof. Dr. : ايهاب سامي

Clinical Diagnosis : Left breast UIQ mass

Nature of Specimen : US guided tru cut biopsy

Receiving Date : 2025-01-11

Analysis ID : 922125

Gender : Female

Patient ID : 687923

PATHOLOGY REPORT

Gross :

Multiple greyish white tissue cores, measured up to 1.6 cm long, totally submitted.

Microscopic :

Serial sections examined from the specimen received revealed cores of tumour tissue formed of malignant ductal epithelial cells, showing marked nuclear pleomorphism and hyperchromasia. There is poor attempt at tubular formation.

There are moderate number of mitotic figures.

There is moderate desmoplasia.

There is peritumoural lymphocytic infiltrate around 10%.

There is moderate necrosis.

There is minimal intraductal component.

Diagnosis :

Left breast mass, US guided tru cut biopsy

, INFILTRATING DUCT CARCINOMA, GRADE III.

- HORMONAL RECEPTORS IN A SEPARATE REPORT.

Prof. Dr. Shady Elia Anis

Shady Elia

Prof. Dr. Elia Anis Ishak

Elia Anis



This report is electronically signed, any abrasion or modification would render it void.

Dr.

Elia Anis Ishak

M. Sc. Ph. D. Pathology - Prof. of Pathology, Cairo University
Fellow, Armed Forces Institute of Pathology, USA
Member, United States Canadian Academy of Pathology

Dr.

Shady Elia Anis

M. Sc. MD Pathology, Ass. Prof. of Pathology, Cairo University
Fellow, Mass General Hospital, Harvard, USA
Member, United States Canadian Academy of Pathology

Patient Name : سميرة موسى عبد الله

Analysis ID : 925303

Age : 55 Y : 6 M : 1 D

Gender : Female

Referred By Prof. Dr. : ايهاب سامي

Clinical Diagnosis : Left multicentric breast masses.

Nature of Specimen : Left modified radical mastectomy.

Receiving Date : 2025-01-22

Patient ID : 387923

PATHOLOGY REPORT

Gross :

Left modified radical mastectomy specimen measured 29x22x4 cm, with covering skin ellipse measured 24x19 cm. Nipple and areola looked unremarkable grossly. Serialing revealed four masses with firm gritty cut section and ill defined borders, measured 2.4x2.2x1.5 cm, 2.3x1.3x1 cm, 1.3x1x0.8 cm and 0.6x0.6x0.5 cm. The least distance between the masses was 2 cm and the largest distance was 8 cm.

The least deep margin was 2 cm away.

Dissection of the axilla revealed 21 lymph nodes, the largest measured 2.5x1.5x0.5 cm.

Microscopic :

Sections examined from specimen received revealed breast tissue showing multifocal (four foci) of tumor tissue.

One focus of them (measuring 2.4x2.2 cm) showing tumor formed of malignant ductal cells with marked nuclear pleomorphism, poor tendency to tubular formation and showing moderate number of mitotic activity.

Wide area of necrosis is seen. Ductal carcinoma insitu less than 5 % (solid pattern).

There is peritumoral lymphocytic infiltrate around 40 %.

The other three foci showing tumour tissue formed of discohesive malignant epithelial cells arranged in single file strands with targetoid pattern and signet ring cell features.

The cells showing eosinophilic cytoplasm and moderately pleomorphic nuclei.

There is moderate number of mitotic figures. There is moderate desmoplasia.

Lympho-vascular space invasion not detected.

Perineural invasion is absent.

There is an insitu lobular component about 10% formed of dilated ducts with malignant lining showing solid pattern.

Sections from the nipple and areola are free.

Surgical margins are free.

Sections examined from the axillary nodes revealed NO metastatic deposits in 21 lymph nodes.



Dr.

Elia Anis Ishak

M. Sc. Ph. D. Pathology - Prof. of Pathology, Cairo University
Fellow, Armed Forces Institute of Pathology, USA
Member, United States Canadian Academy of Pathology

Dr.

Shady Elia Anis

M. Sc. MD Pathology, Ass. Prof. of Pathology, Cairo University,
Fellow, Mass General Hospital, Harvard, USA
Member, United States Canadian Academy of Pathology

Patient Name : سميحة موسى سعد الله

Age : 55 Y : 6 M : 1 D

Referred By Prof. Dr. : ايهاب سامي

Clinical Diagnosis : Left multicentric breast masses.

Nature of Specimen : Left modified radical mastectomy.

Receiving Date : 2025-01-22

Analysis ID : 925303

Gender : Female

Patient ID : 387923

PATHOLOGY REPORT

Diagnosis :

Left modified radical mastectomy, MULTIFOCAL BREAST MASSES,

- ONE FOCUS SHOWING INFILTRATING DUCT CARCINOMA GRADE 3, DUCTAL CARCINOMA INSITU (LESS THAN 5 %).

- THREE FOCI SHOWING INFILTRATING PLEOMORPHIC LOBULAR CARCINOMA, GRADE III, INSITU LOBULAR COMPONENT 10%.

- FREE SURGICAL MARGINS, NO AXILLARY LYMPH NODE METASTASIS (L.N. 0 / 21).

* TUMOR STAGE: pT2(m), pN0.

Prof. Dr. Shady Elia Anis



Prof. Dr. Elia Anis Ishak



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Elia Anis Ishak

M. Sc, Ph. D, Pathology - Prof. of Pathology, Cairo University
Fellow, Armed Forces Institute of Pathology, USA
Member, United States Canadian Academy of Pathology

Dr.

Shady Elia Anis

M. Sc, MD Pathology, Ass. Prof. of Pathology, Cairo University
Fellow, Mass General Hospital, Harvard, USA
Member, United States Canadian Academy of Pathology

Patient Name : سميرة موسى سعد الله

Age : 55 Y : 6 M

Referred By Prof. Dr. : ايهاب سامي

Clinical Diagnosis : Left breast UIQ mass

Nature of Specimen : Block (922125)

Receiving Date : 2025-01-11

Analysis ID : 922126

Gender : Female

Patient ID : 687923

IMMUNOHISTOCHEMISTRY REPORT

Technique :

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain.

The sections were treated against:

- Estrogen receptors - Progesterone receptors - Her 2 - Ki67

Results :

80 % of tumor cells show STRONGLY POSITIVE Staining for Estrogen receptors.
5 % of tumor cells show WEAKLY POSITIVE Staining for progesterone receptors.
Sections treated for HER2 revealed NEGATIVE CELL MEMBRANE STAINING.
18 % of Tumour cells are POSITIVE for Ki67.

Conclusion :

Block, TUMOUR CELLS ARE
POSITIVE FOR ESTROGEN RECEPTORS (SCORE 8 / 8).
POSITIVE FOR PROGESTERONE RECEPTORS (SCORE 3 / 8).
HER 2 NEGATIVE, SCORE 0.
THE PROLIFERATION INDEX OF Ki67 IS 18 %.

Prof. Dr. Shady Ella Anis

Shady Ella Anis

Prof. Dr. Elia Anis Ishak

Elia Anis



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Name: Samiha Mousa Saad Allafi
Referred by: Prof.

ID: 4226610

Date: 02/01/2025

Thanks for reference

Bilateral Digital Mammography & High Resolution Breast Sonograph

Revealed :

- Scattered fibro glandular densities (ACR B).
- Left UIQ rounded speculated dense lesion is seen about 4 cm from the nipple.
- Left UOQ small irregular speculated lesion with architectural distortion.
- Minimal skin thickening and mild nipple retraction.

Ultrasound:

- Heterogeneous fibro glandular mammary parenchyma.
- **Left UOQ 2:00 mid zonal irregular dense lesion with indistinct margins distortion and surrounding desmoplastic reaction seen measuring about 24x16x17 mm. with small nearby satellite measuring about 2.3x2.3 mm.**
 1. Another area of altered parenchyma is seen inline with the nipple about 12 mm from the fore mentioned lesion and about 1 mm from the nipple in the para areolar region measuring about 83x7x11 mm.
 2. Suspected Intra ductal extension.
- **Left UIQ 11:00 mid zonal rounded hypo echoic lesion with irregular speculated margins and multiple small surrounding satellites, seen measuring about 32x30x29 mm.**
 1. It is seen surrounding by desmoplastic reaction,
 2. It shows surrounding inflammatory changes of the breast with echogenic fat & mild interstitial edema.
- Mildly dilated retro areolar ducts.
- No architecture distortion.
- Left axillary solitary suspicious lymphnode seen measuring about 22x7 mm with focal cortical thickening measuring about 4 mm.
- No pathologically enlarged infra or supra clavicular lymphnodes.



S C A N

OPINION :

- Left breast multi-centric malignant looking lesions, with suspicious left axilla lymphnode. BIRADS 5.
- Dynamic MRI of the breast is recommended to assess the full extent of the disease.
- Tissue biopsy for histo pathology from both lesions & immune histochemistry are recommended.

Best regards
Dr. Yasmin Essam Khalifa, M.

Defining BIRADS classification of breast lesions according to the American Collage of Radiology:

<u>Category</u>	<u>Suspected Cancer Risk</u>
BIRADS 0 : Non-informative study.	
BIRADS 1 : Normal study.	0 %
BIRADS 2 : Benign findings.	0 %
BIRADS 3 : Probably benign.	2 %
BIRADS 4 : Probably malignant.	50 %
A : mild suspicion.	
B : moderate suspicion.	
C : high suspicion.	
BIRADS 5 : Malignant.	98-100 %
BIRADS 6 : Pathologically proven malignant lesion.	



SCAN O LABS

Name : Samiha Mousa Sadallah Iskander

Branch : Mandi

Ref.Physician Prof : Ehab Sami

Date : 2025-01-20

Medical No. : 110819908

Ins.Company : Walk-In

Whole body PET/CT EXAMINATION:

Clinical History:

- o 53 years old patient diagnosed with Lt breast IDC.
- o (No available previous radiological data).
- o PET/CT requested for initial staging assessment.

Technique:

- o For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCI was performed using GE Discovery IQ simultaneously and used for attenuation correction, anatomical localization and diagnosis. Max. variant of SUV; a Semi-quantitative analysis would be done for selected ROIs and the normal threshold is < 2.5 .

Report:

No previous PET-CT available for comparison.

Hepatic Reference SUVmax~3.3

Ill-defined spiculated FDG avid Lt breast UIQ parenchymal soft tissue density lesion is seen measuring 2.1 x 2 cm with SUVmax~4, likely neoplastic for correlation with histopathological/IHC data.



Unremarkable Rt breast with no FDG avid focal pathological lesion detected.

Bilateral small non FDG avid axillary LNs, mostly with preserved fatty hilum, the largest at the Lt side reaching ~1.1 cm.

Normal tracer uptake over brain, myocardium and renal system.

Head and Neck:

- * Normal diffuse brain activity with no hypermetabolic brain lesions.
- * Physiologic FDG uptake is seen in the nasopharynx, oropharynx, salivary glands and larynx, showing normal appearance.
- * Physiologic FDG uptake is seen in submandibular and parotid salivary glands, showing normal appearance.
- * Low grade diffuse FDG uptake is seen over thyroid gland, more appreciated at Rt thyroid lobe with SUVmax~3.3, likely thyroiditis, for further clinico-laboratory correlation.
- * Normal appearance of the neck muscles & vessels.
- * No hypermetabolic cervical lymphadenopathy detected apart from few sub-cm non FDG avid Lt supra-clavicular LNs, the largest reaching ~7 mm.
- * No FDG avid neck soft tissue mass lesions.

Chest:

- * No hypermetabolic mediastinal lymphadenopathy.
- * No FDG avid lung nodules or masses detected.



SCAN 9 LABS

مركز البحوث والتشخيص

Name : Samiha Mousa Sadallah Iskander

Branch : Maadi

Ref.Physician Prof : Ehab Sami

Date : 2025-01-20

Medical No. : 110819908

Ins.Company : Walk-In

- * No metabolically active pericardial or pleural lesions detected.
- * Physiologic FDG uptake is noted in the heart, showing normal shape.

Abdomen and Pelvis:

- * Heterogenous FDG uptake is noted over ascending colonic segment with SUVmax~6.3, for clinical and/ or colonoscopy assessment.
- * Physiologic diffuse FDG uptake is seen over average sized liver with homogenous parenchymatous texture. No FDG avid focal lesions detected. Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries. No intra-hepatic or extra-hepatic biliary radicle dilatation.
- * Physiologic diffuse FDG uptake is seen over average size spleen with homogenous parenchymal texture and no FDG avid focal lesions seen.
- * Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes. Physiologic FDG excretion is seen in the kidneys and bladder.
- * Normal appearance of the pancreas, aorta, I.V.C and both supra-renals with no FDG avid lesions detected.
- * No hypermetabolic abdominal or pelvic lymphadenopathy.
- * No FDG avid ascites or abdominal collections noted.

Musculoskeletal:

- * Normal FDG activity is seen over the axial skeleton. No focal active blastic or lytic bone lesions detected.
- * Diffuse non FDG avid sclerotic changes are noted over both pubic bones.
- * Spondylo-degenerative changes of the spine.

Conclusion:

Positive study for Lt breast UIQ FDG avid soft tissue neoplastic lesion, for correlation with hsto-pathological and IHC data.
No active distant metastatic deposits.

Dr. Nada Fadl
Nuclear Medicine

Thank you for referral

Dr. Walid Omar, MD.
Prof. of Nuclear Medicine

Patient's Name: Samihah Musa Saad Allah Iskander
Date: 2026-08-26
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTISLICE POST-CONTRAST CT EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS

Clinical history: Known case of carcinoma left breast, status post modified radical mastectomy.

CHEST:

- No pulmonary nodules, masses, or cavitary lesions.
- No consolidation or collapse.
- Diffuse mosaic attenuation is seen throughout both lung fields, ? small airways disease — clinical correlation advised.
- No hilar or mediastinal masses or lymphadenopathy.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

ABDOMEN AND PELVIS:

- Liver: Average size, homogeneous parenchymal attenuation, no focal lesion or biliary dilatation.
- Kidneys: Normal site and size, smooth cortical outlines, preserved parenchymal thickness, no stones or obstruction.
- Spleen, pancreas, adrenal glands, aorta, and IVC: Normal.
- Urinary bladder: No masses, diverticula, or stones.
- Pelvic organs: Normal appearance, no masses or collections.
- Few sub-centimetric bilateral external iliac lymph nodes noted.
- No ascites.

Opinion:

- *No pulmonary or hepatic metastatic lesions.*

Dr.sherif Wagdy

Dr.sherif Wagdy

1216

Patient's Name: Samihah Musa Saadallah Iskander
Date: 2026-05-14
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt



RIGHT DIGITAL SOFT TISSUE MAMMOGRAPHY WITH HIGH RESOLUTION SONOGRAPHY REVEALED:

CLINICAL SUMMARY:

Left MRM

FINDINGS:

RIGHT BREAST:

Fairly dense fibro-glandular densities, (ACR b).
Retroareolar tubular structures are seen, with no spiculations, masses, asymmetry, distortion or suspicious microcalcifications.
Complimentary **US** demonstrated dilated ducts with one exhibiting echogenic content, likely inspissated secretion, and a caliber reaching 5.5mm. Colour Doppler depicted no abnormal flow.
Normal skin/nipple/areola complex.
Non-specific axillary nodes.

LEFT MASTECTOMY BED.

Clear mastectomy bed with no masses or fluid collections.
No pathological regional lymph nodes.

CONCLUSION:

- Right breast (BIRADS 2): Duct ectasia
- Clear left mastectomy bed

Best regards,
Dr. Mahi Mokhtar